

Improving Adherence in Patients Taking Cycle-directed Oral Anti-cancer Medications



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Background

- Adherence is the degree or extent to which a patient acts in accordance with prescribed intervals and doses of a medication regimen
- Benefits of patient-administered oral anti-cancer medications (OAMs) are balanced by challenges in maintaining adherence:
 - Complex dosing regimens, toxicity/adverse effects, drug interactions, monitoring
- Consequences of non-adherence include lower drug efficacy leading to potentially worse outcomes such as decreased survival
- Studies have shown adherence aids such as treatment calendars, one-on-one education, and pill boxes improve adherence in patients taking OAMs
 - Curry et al. found an increase in adherence from 30% to 85% over 2 years
- Mizzou Specialty Pharmacy recognized non-adherence in cycle-directed OAMs and wanted to conduct a quality improvement project to improve patient care
- Mizzou Specialty Pharmacy pharmacists and technicians collaborate to provide refill assessments, one-on-one treatment counseling, and follow up assessments, allowing for easy education and monitoring of cycle-directed OAMs
- Additionally, due to the limited study of cycle-directed OAM treatment calendars in oncology patients, further research is needed

Purpose

To improve adherence in patients taking cycle-directed OAMs at Mizzou Specialty Pharmacy

Methods

Design & Plan

- Single center, observational study
- Patients provided a treatment calendar with each refill including start/stop dates & check boxes for dose reminders
- Treatment calendars scanned into the electronic medical record for provider reference
- Adherence assessed via patient recollection, cycle date confirmation, and number of pills left at time of assessment
- At each adherence check, patients gave feedback on opinion & use of calendars
- Data collected from September 2023 to September 2024

Inclusion Criteria	Exclusion Criteria
Age ≥18 years old	Pregnant
Prescribed one of the following by Mizzou Specialty Pharmacy provider -Ibrance (palbociclib) -Inqovi (decitabine/cedazuridine) -Kisqali (ribociclib) -Lonsurf (trifluridine/tipiracil) -Temodar (temozolomide) -Xeloda (capecitabine)	Prisoner Patient received medication for < 3 refills/cycles within the 12-month study period
Filled at least 3 refills of OAM with Mizzou Specialty Pharmacy	

Primary Outcome

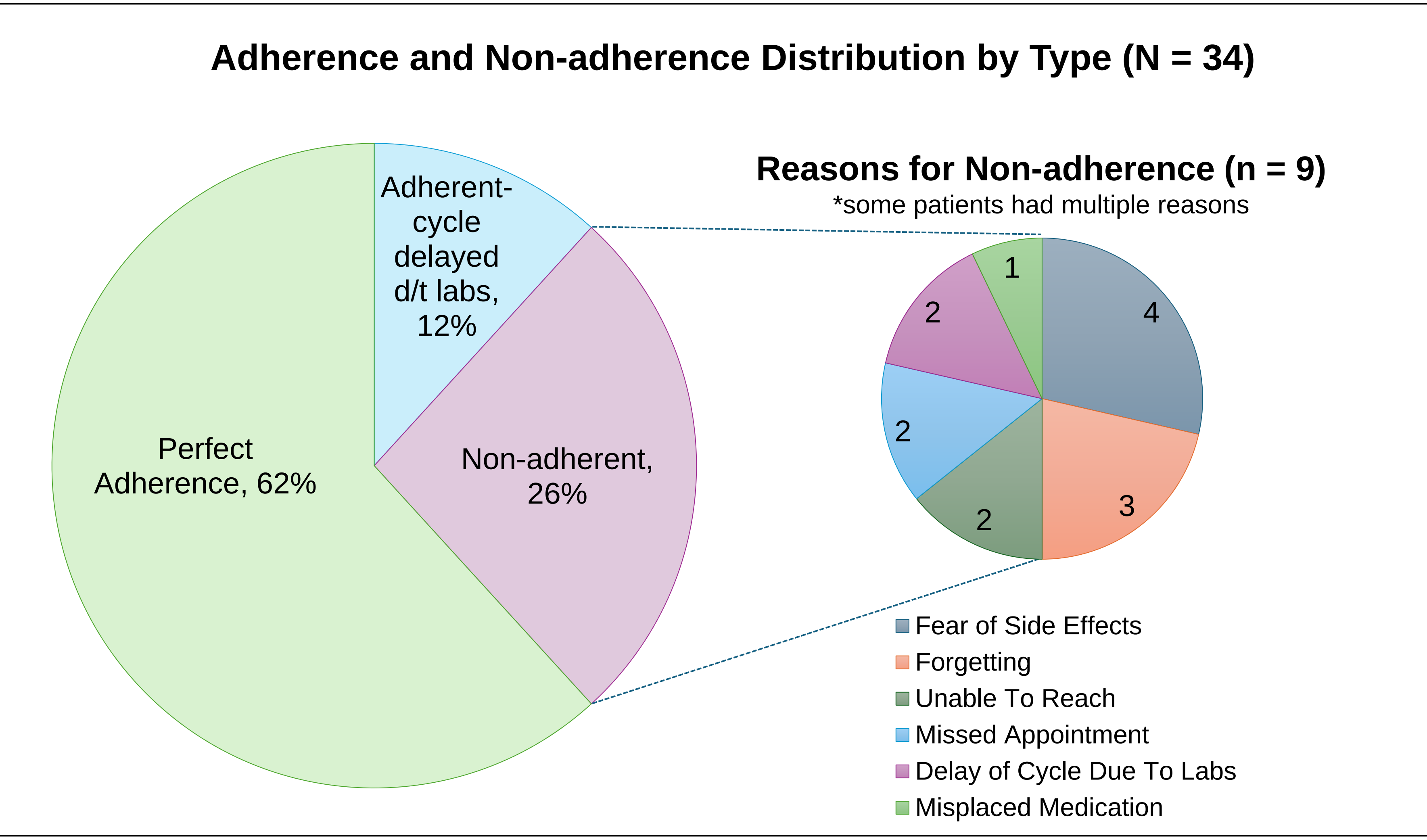
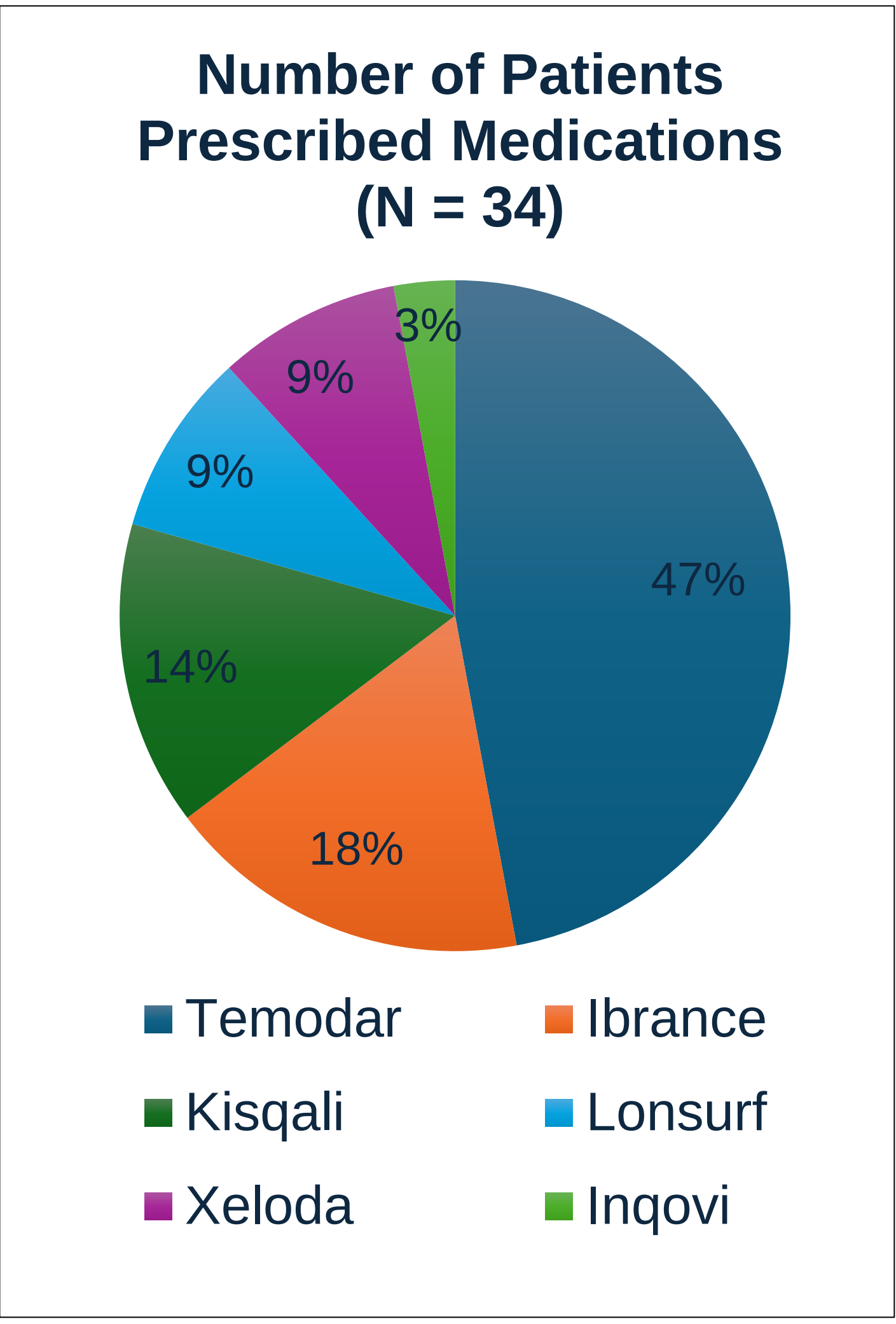
- Number of patients who remained adherent on cycle-directed therapy after receiving a treatment calendar with each refill

Secondary Outcome

- Reasons for non-adherence

Results

Results (N = 34)
74% (n= 25) of patients were adherent to their therapy regimen after receiving treatment calendars Of the 25 adherent patients, 4 were adherent but had delays to their treatment due to poor labs (provider-directed hold)
Of the 26% (n= 9) of non-adherent patients, the most common reason for non-adherence was fear of adverse effects or forgetting/not using calendar
Average number of cycles included for each patient = 6.67 cycles
97% (n=33) patients stated the calendars were helpful for adherence



Conclusion

Strengths

- This study's results had similar results to literature in terms of predicted outcomes

Limitations

- Fewer number of patients due to implementation for only six medications
- Lack of ability to compare pre/post data

Conclusions

- Adherence was achieved in majority of patients who used treatment calendars
- Fear of side effects and forgetting were the top reasons for non-adherence
- Patients and providers were satisfied with adherence aids provided

Next Steps

- Continuing to expand the project by providing treatment calendars for all cycle-directed therapies provided at Mizzou Specialty Pharmacy
- Expand pharmacist counseling to include in-depth review of the calendars upon receipt
- Expand provider & clinic education to review treatment calendar location in the EMR to improve scheduling errors

References

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Disclosure

The authors of this presentation have no financial or personal relationships with commercial entities that may have a direct or indirect interest in the subject matter of this presentation.